A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

13 May 2026

Dr Tara Thyagarajan
Junction Place Medical Centre

Dear Dr Thyagarajan,

Re: Ms Alice Toleman
DOB: 11/03/1994 Phone: 0437 351 136
Address: 18A Kingsley Street, CAMBERWELL, VIC, 3124
Email: alicectoleman@gmail.com

Summary

- **Complex post-bariatric symptoms including suspected dumping syndrome and post-prandial tachycardia.**
- **Avoidant eating patterns and significant medical anxiety.**
- **Commencement of low-dose propranolol for symptomatic tachycardia.**
- **Rationalisation of specialist care to focus on nutritional intake and mental health.**
- **No action required by you.**

I had the pleasure of reviewing Alice in the rooms today. As you are aware, Alice has a complex medical history involving bariatric surgery, endometriosis, and multiple sclerosis. She is currently managing several concurrent health issues, including recent findings of adenomyosis and a breast lump with nipple discharge, the latter of which she is investigating with an endocrinologist and breast surgeon.

From a gastrointestinal perspective, her bowel habit has improved significantly, which she attributes to her amitriptyline and increased water intake. However, she remains troubled by nocturnal reflux and regurgitation, requiring her to sleep upright. She experiences vomiting two to three times per month, typically described as food boluses becoming "stuck". Interestingly, she experiences hiccups with every meal.

Of most concern is her reaction to food. She describes significant post-prandial fatigue and palpitations, with her heart rate reaching 120 bpm after small meals. This presentation, combined with suspected insulin spikes from dumping syndrome, has led to a fear-based avoidant eating pattern. She currently avoids eating until late in the evening to maintain functionality during the workday. We discussed that her relationship with food is increasingly complicated, bordering on an eating disorder, which is being addressed by her psychiatrist, Aileen. She has ceased her GLP-1 agonist due to its negative impact on her mood.

Impression and Plan

I have suggested to Alice that we attempt to rationalise her medical care. Given the sheer volume of specialists and investigations she is currently navigating, her medical anxiety is quite high. I believe our primary focus should be on her nutritional intake and her physiological response to eating.

I have asked Alice to trial "exposure therapy" by consuming small amounts of protein in the morning, even on weekends, to attempt to retrain her brain’s response to food and mitigate the fatigue. To address the

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post-prandial tachycardia, I have commenced a trial of low-dose propranolol (10 mg) to be taken before meals. I have advised her to monitor her blood pressure and heart rate given her history of normal-to-low readings.

She will continue her amitriptyline (25 mg nocte) and pantoprazole (40 mg BD). I have encouraged her to follow through with the endocrinology review, as they may consider a mixed meal test or continuous glucose monitoring to formally evaluate her post-prandial insulin response. We will push the investigations regarding her pelvic floor and other minor symptoms to the background for now. I will review her again in three months' time.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Aileen Shuey

All appointments and enquires
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Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065